Responding to the chikungunya virus in west Africa

Chikungunya, a neglected viral disease transmitted by *Aedes* mosquitoes, which also carry dengue and yellow fever, has been identified in more than 110 countries across Asia, Africa, Europe, and the Americas.[1] Although chikungunya typically results in low mortality, the disease can lead to prolonged illness and, in rare instances, complications, such as neurological disorders, eye inflammation, and heart issues.[2] No specific treatment or vaccine is available, making prevention and symptomatic treatment the primary management strategies.

From 2017 to 2022 we have seen a resurgence of chikungunya in various regions.[2] In 2023, more than 460 000 cases and 350 deaths were reported globally, with the highest burden of the disease in South and central America.[2] This resurgence is also notable in west Africa, with 565 confirmed cases across Senegal, Burkina Faso, Mali, The Gambia, and Guinea by November, 2023. The re-emergence in regions such as southeast Senegal (Kédougou), paralleling the biggest rise in local temperatures between the 1980s and 2023 and concurrent dengue outbreaks, suggests environmental, socioeconomical, and anthropical factors contributing to virus spillover from sylvatic to urban cycles. The spread of *Aedes albopictus,* possibly aided by climate change, increasing trade, uncontrolled urbanisation, and increased global connectivity,[3] has been a contributing factor, as shown in past outbreaks in Cameroon, Gabon, and the Democratic Republic of the Congo.[4]

Addressing chikungunya in west African countries, which vary in resources and infrastructure, presents several challenges. These challenges include poor access to diagnostics and inadequate funding for research and development. Resolving many of these issues might require additional support and collaboration from the international community. To address these issues, promoting domestic funding for sustainability and enhancing innovation and health technology access through the development of digital platforms and integrated case management strategies is crucial. This effort can include mapping data from endemic countries. Training local clinicians, investing in public health laboratories and local manufacturing hubs, implementing genomic surveillance, and developing and delivering innovative and affordable point-of-care solutions are also key steps. These measures, aligned with the One Health approach, can improve community-based surveillance and risk assessment, fostering cross-country collaboration and health sector cooperation.

Another key aspect is the accelerated confirmation of clinical trials for potential vaccines, such as the IxChiq vaccine (Valneva, France), which received US Food and Drug Administration approval after phase 3 trials.[5] Supporting vaccine manufacturing in low-income and middle-income countries is essential for a rapid and equitable response to future outbreaks. This collective approach, embracing technology, health-care infrastructure improvements, and international cooperation, is key for managing chikungunya effectively and bolstering global health resilience.

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Cofinancing immunisation through national health insurance

Sustainable immunisation financing is a global health issue of utmost concern that requires serious political attention and commitment from all countries. However, the efforts that many low-income and middle-income countries (LMICs), with persistently underfunded immunisation systems, are making to institute policy reforms that can stimulate more domestic immunisation financing are alarmingly passive. The COVID-19 pandemic exposed the gross level of unpreparedness of many countries to meet the needs of their people during external shocks (eg, unprecedented health emergencies), thus underscoring the importance of adequately funded and self-reliant health systems. Yet, many countries continue to depend on donor funds for routine immunisation, even in the post-pandemic period, without any tangible and viable action plan to transition away from this support. Many of the LMICs that are funded by Gavi, the Vaccine Alliance, are either in the preparatory or accelerated transition phase and, if that period ends, they will need to become fully self-financed.[1]

The possibility of securing higher allocations for immunisation in national budgets is currently capricious because countries are confronting an unprecedented level of